

Other Antithrombotics for Secondary Stroke Prevention

	CSPS.com (2019)	OCEANIC-STROKE (2026)
Drug class	PDE3 inhibitor (cilostazol)	Factor XIa inhibitor (asundexian)
Population	High-risk noncardioembolic ischemic stroke, 8–180 days post-onset (Japan)	Noncardioembolic stroke (NIHSS ≤15) or high-risk TIA (ABCD2 6–7), ≤72h (37 countries)
N	1,879 (ITT)	12,327
Design	Open-label RCT	Double-blind, placebo-controlled RCT
Intervention	Cilostazol 100 mg BID + aspirin or clopidogrel	Asundexian 50 mg daily + planned SAPT/DAPT
Control	Aspirin or clopidogrel alone	Placebo + planned SAPT/DAPT
Follow-up	Stopped early for slow recruitment (median 1.4 yr)	Median 567 days (~1.6 yr)
Primary endpoint	Recurrent ischemic stroke	Ischemic stroke
Primary result	HR 0.49 (0.31–0.76); 2.2%/yr vs 4.5%/yr	HR 0.74 (0.65–0.84); 6.2% vs 8.4%
Key secondary	Stroke, MI, vascular death: HR 0.52 (0.35–0.77); 38 (4%) vs 78 (8%)	CV death, MI, or stroke: HR 0.83 (0.74–0.92); 9.2% vs 11.1%
Bleeding / Safety	Severe bleeding: HR 0.66 (0.27–1.60); 0.6%/yr vs 0.9%/yr	ISTH major bleeding: HR 1.10 (0.85–1.44); 1.9% vs 1.7%
Strengths		
Limitations	Open-label design. Underpowered: stopped early at 47% of target enrollment. Generalizability: exclusively Japanese cohort. Tolerability: high early dropout in dual-therapy arm (23% vs 14%) due to headaches and palpitations.	High dropout: ~26% permanently discontinued trial drug (balanced across arms). Generalizability: excluded severe strokes (few with NIHSS ≥8); very low enrollment of Black patients (2.3%) and TIA patients (~5%).
Clinical implications	Highly efficacious option for long-term stroke prevention in Asian populations, but confidence weakened by early trial stoppage, open-label bias, and poor drug tolerability.	

CSPS.com: Toyoda K et al. *Lancet Neurol* 2019;18:539-548. OCEANIC-STROKE: Sharma M et al. *NEJM* 2026;394:1467-1479.

Source-qualified teaching update | 30 September 2026

OCEANIC-STROKE required at least one of nonlacunar infarct, history of atherosclerosis or cerebrovascular atherosclerotic plaque. Major bleeding 1.9% versus 1.7%, HR 1.10 (95% CI 0.85-1.44), does not establish absence of every bleeding effect. CSPS.com required MRI-confirmed high-risk noncardioembolic stroke with >=50% intracranial OR extracranial stenosis, OR at least two specified risk factors; ICAD was not universal.

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